WOMEN'S HEALTH SERIES

The Indian PCOS Playbook

2026-27 Edition

Understand it. Get the right tests. Manage it with food, movement and medicine — a calm, evidence-based system for Indian women, built on WHO, ICMR, FOGSI and the 2023 International Evidence-Based PCOS Guideline.

THE COMPLETE INDIAN PCOS PLAYBOOK

Symptoms | Diagnosis | Indian Diet | Movement | Mental Health | Treatment | Fertility

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Please read this first

This playbook is education, not medical advice, and it is not a diagnosis. PCOS is a medical condition that has to be assessed and managed by a qualified doctor, usually a gynaecologist, an endocrinologist or your family physician. Nothing in this book replaces a consultation, a physical examination or a blood test.

Do not start, stop or change any medicine, supplement or dose based on this book. The treatment section explains what your doctor may consider and why, so you can have a better conversation with them — not so you can self-prescribe. If you are pregnant, planning a pregnancy, breastfeeding, under 18, or living with diabetes, high blood pressure or another condition, your plan must be set by your doctor.

The facts here are drawn from official and peer-reviewed sources — the World Health Organization, the ICMR national PCOS task-force study, FOGSI's clinical recommendations for India, and the 2023 International Evidence-Based Guideline. Where the evidence is uncertain, this book says so. Your body is specific to you, and so is your care.

A note on language. This book uses the word "women", following the source material, to include all girls and gender-diverse people who have ovaries and may develop PCOS. PCOS does not discriminate by identity, and neither should good information.

Seek care promptly if you have very heavy or non-stop bleeding, bleeding between periods, sudden severe pain in the lower abdomen, rapid voice deepening or clitoral enlargement, or thoughts of harming yourself. These are not things to manage with a diet plan.

Sources are listed in full at the end of the book. Figures are current to October 2026 and rounded for readability.

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How to use this book: read sections 1 to 7 once to understand what is happening in your body. Then live in sections 8 to 14, which are the day-to-day system. Use section 23 at your next appointment, and section 25 to start this week. You do not need to do everything at once — pick the smallest change you can keep.

1. What PCOS really is — and why "PCOD" confuses everyone

PCOS — polycystic ovary syndrome — is a common hormonal and metabolic condition. In simple terms, the ovaries and the adrenal glands make slightly more androgen (the so-called "male" hormones) than usual, the body's cells respond poorly to insulin, and ovulation becomes irregular. The World Health Organization calls it one of the most common hormonal disorders in women of reproductive age and the most common cause of ovulation problems worldwide.

The name is genuinely misleading, and that is worth understanding before anything else. "Polycystic" sounds like you have cysts, but the small bumps seen on an ovary scan are not true cysts — they are immature follicles, each holding an egg that did not mature enough to be released. Many women with PCOS have perfectly normal-looking ovaries on scan, and many women with bumpy ovaries do not have PCOS at all. This is why researchers have long argued the name should change; some health bodies now use the clearer term PMOS (polyendocrine metabolic ovarian syndrome).

So what is "PCOD"?

In everyday Indian conversation, and even in some clinic rooms, people say "PCOD" — polycystic ovarian disease. It is not a formal diagnosis in the international guidelines. The recognised condition is PCOS. In practice, "PCOD" is often used loosely for anyone whose scan shows multiple follicles, even if their periods are regular and their hormones are normal. If you have been told you have "PCOD" but your cycles are regular and your androgen levels are normal, that is not automatically PCOS — and it is worth asking your doctor to be precise about which criteria you meet.

The one-line version: PCOS is a real, diagnosable condition with defined criteria. "PCOD" is a loose label. Ask your doctor to tell you, in writing, which criteria you meet — that single question changes how seriously (or not) you should treat it.

Three things are true of PCOS

- **It is common and it is not your fault.** It runs in families and has a strong genetic component. It is not caused by one bad diet, one stressful year, or a lack of willpower.
- **It is lifelong but very manageable.** There is no cure, but lifestyle, and where needed medication, reduce symptoms and protect your long-term health.
- **It is a whole-body condition, not just a period problem.** It affects metabolism, mood, skin, hair and fertility — which is why this book treats food, movement, sleep, stress and medicine as one system.

It helps to think of PCOS as a dial rather than a switch. At one end, a woman has irregular cycles and a scan finding but feels well. At the other, she has several symptoms at once and clear metabolic changes. Treatment moves the dial — it rarely turns the condition off completely, and that is normal and fine.

2. How common PCOS is in India — the real numbers

PCOS is not a rare condition you have to hunt for. It is one of the most common hormonal conditions in Indian women, and the country's own national study confirmed it.

19.6%
Indian women
Prevalence by Rotterdam criteria — the ICMR national task-force study

7.2%
By strict criteria
Prevalence by the stricter NIH criteria, same study

1 in 5
Adolescent girls
Pooled estimate for Indian adolescents (FOGSI)

70%
Undiagnosed
Global estimate of women with PCOS who do not know they have it (WHO)

The Indian Council of Medical Research ran the country's largest study of PCOS, screening nearly 10,000 women aged 18 to 40 across five zones. It found a national prevalence of 19.6% using the Rotterdam criteria that most doctors use, and 7.2% using the stricter NIH criteria — the difference simply reflects how strict the definition is. The WHO puts the global figure at 10 to 13% of reproductive-aged women.

What makes PCOS especially important in India is not just how common it is, but how often it travels with other metabolic problems. In the ICMR study, among the women who had PCOS:

Companion condition	Share of Indian women with PCOS
Dyslipidaemia (abnormal cholesterol)	91.9%
Obesity	43.2%
Fatty liver (NAFLD)	32.9%
Metabolic syndrome	24.9%
Hypertension	8.3%
Type 2 diabetes	3.3%

Read that again: nine in ten Indian women with PCOS already have abnormal cholesterol, and about one in three has fatty liver. This is the reason PCOS deserves attention even when your periods are the only thing that bothers you. The condition is quietly raising your risk of diabetes and heart disease years before you feel anything.

There is also a regional pattern. The ICMR study found the highest PCOS prevalence in central and northern India, and lower rates in the northeast. India's separate national diabetes study (ICMR-INDIAB) found metabolic disease climbing fastest in the south — with the diabetes-to-prediabetes ratio in several states showing that many people are one step away from diabetes. Put together, these numbers say something important: PCOS in India is not an isolated hormone problem, it is a marker of a broader metabolic risk that the whole family often shares.

What this means for you: if you have PCOS, your annual check should include blood sugar, cholesterol and blood pressure — not just a period review. Section 23 gives you the exact list.

3. The symptoms: a self-check map

PCOS looks different in different women. Some have almost no symptoms; others have several at once. Symptoms often begin around the first period and change over the years. Use the map below as a prompt for a doctor's visit — not as a diagnosis.

What you might notice	What it usually means in PCOS
Periods that are far apart, missed, or absent	Ovulation is not happening regularly (oligo-anovulation).
Very heavy, long or painful bleeding	The womb lining building up without regular shedding.
Extra hair on face, chin, chest, belly or thighs	Hirsutism — a visible sign of higher androgens.
Thinning hair on the scalp, or a widening parting	Female-pattern hair loss, also driven by androgens.
Persistent acne, especially on jaw, chest and back	Androgen effect on oil glands; often resists normal creams.
Dark, velvety skin on neck, armpits or groin	Acanthosis nigricans — a marker of insulin resistance.
Weight gain, especially around the belly, hard to shift	Insulin resistance and central fat storage.
Low mood, anxiety, feeling unlike yourself	Psychological features are common and part of PCOS.
Snoring, waking unrefreshed, daytime tiredness	Possible sleep apnoea, which is more common with PCOS.

A single one of these on its own does not mean PCOS. Acne alone is a weak predictor; hair loss alone is a weak predictor. It is the combination — usually irregular cycles plus signs of high androgens — that points towards it.

One symptom is easy to overlook and worth naming: acanthosis nigricans, those soft dark velvety patches on the neck, underarms or groin that some people mistake for dirt. Indian guidelines count it as an additional clue for PCOS, because it signals insulin resistance. If you have been scrubbing at a patch like this for years, mention it at your appointment.

Track your own pattern before the appointment

For three months, note the first day of every period, how many days it lasts, and whether it is light, normal or heavy. Note any unusual hair growth or loss, acne, and your energy and mood. A simple notes app or a paper calendar is enough. Your pattern over time is far more useful to a doctor than a single month's memory.

If you take the combined pill, remember it makes your natural cycle invisible — the bleed you get is a withdrawal bleed, not a period. If your doctor needs to assess your natural cycles or androgens, they may ask you to stop the pill for a period first, with alternative contraception in the meantime.

4. How PCOS is diagnosed in India — the tests to ask for

There is no single blood test that says "PCOS". In adults, doctors in India use the Rotterdam criteria: PCOS is diagnosed when you have at least two of three things, once other causes are ruled out. FOGSI, the body representing Indian gynaecologists, recommends exactly this approach for Indian women.

The three criteria

- **Irregular cycles or ovulation problems.** From three years after your first period to menopause, cycles shorter than 21 days, longer than 35 days, or fewer than eight per year.
- **Signs of high androgens** — either visible (hirsutism, acne, scalp hair loss) or on a blood test (raised testosterone).
- **Polycystic ovaries on an ultrasound** — many small follicles (20 or more in one ovary) or an enlarged ovary (10 ml or more).

If you clearly have two of these, you do not need the third confirmed. In adolescents, the rules are stricter and an ultrasound is not used — see section 22.

The tests worth asking about

Test	Why it is done
Total testosterone / free androgen index	To confirm high androgens, especially if visible signs are mild.
TSH (thyroid)	Thyroid problems mimic PCOS and must be excluded.
Prolactin	A raised level is another PCOS look-alike.
FSH and LH	To assess the ovary's signalling and exclude other causes.
AMH (optional)	FOGSI notes a level of 3.2 ng/mL or more can suggest polycystic ovaries, where ultrasound is unclear. Not for adolescents.
75g oral glucose tolerance test (OGTT)	The most accurate way to check blood sugar and insulin resistance.
Fasting glucose and HbA1c	Used if an OGTT is not possible.
Fasting lipid profile	Recommended for every woman with PCOS at diagnosis.
Blood pressure	Checked at every visit.
Pelvic ultrasound	In adults, to look at follicle number and ovary size. Transvaginal is most accurate if acceptable.

Ask for the numbers, not just a yes or no. Request a copy of your reports. Knowing your actual fasting glucose, HbA1c, cholesterol and testosterone lets you track change over a year — and tells you whether your plan is working.

One more thing: a scan showing "multiple follicles" is extremely common and, on its own, proves nothing. In healthy regularly-cycling women, up to a quarter to a third can show this pattern. This is exactly why the loose label "PCOD" causes so much needless worry.

5. The four phenotypes: your PCOS may look different

Because PCOS is diagnosed from three features, women end up with different combinations. Doctors call these phenotypes, and they matter because your symptoms and your risk profile depend on which one you have.

Phenotype	What is present	What it tends to mean
A	High androgens + irregular cycles + polycystic ovaries	The classic picture; often the most visible symptoms.
B	High androgens + irregular cycles	Symptoms driven mainly by androgens and cycle problems.
C	High androgens + polycystic ovaries	Androgen signs with relatively regular cycles.
D	Irregular cycles + polycystic ovaries	Cycle and metabolic problems without clear androgen signs.

In the ICMR national study, phenotypes C and D together made up about two-thirds of cases in India — that is, many Indian women with PCOS do not fit the "extra hair and acne" stereotype at all. If your doctor says you have PCOS but you look nothing like the textbook description, that is normal, not a mistake.

Phenotype also shapes what treatment helps most. If your main issue is irregular cycles, the priority is cycle regulation and protecting the womb lining. If it is metabolic — weight, fatty liver, blood sugar — the priority is insulin sensitivity. If it is cosmetic — hair and acne — the priority is lowering androgens. Most women need a mix, but knowing your dominant phenotype tells you where to put your first effort.

Why this matters: ask your doctor which phenotype you have. Phenotype D, for example, is more about cycle and metabolic health than about hair or acne — so a plan that focuses only on cosmetic symptoms would miss the point for you.

6. Insulin resistance: the engine under the hood

If you understand one mechanism in PCOS, make it this one. Insulin is the hormone that moves sugar from your blood into your cells. In insulin resistance, your cells stop responding well, so your body makes more and more insulin to compensate. High insulin then pushes the ovaries to make more androgens, and drives fat storage around the belly. That is the loop that connects irregular periods, weight gain and skin and hair symptoms.

Indian women with PCOS carry a particularly heavy version of this loop. FOGSI reports insulin resistance in 50 to 75% of Indian women with PCOS — a higher share than in many Western populations at the same body weight. This is why South Asian women can develop metabolic problems at a lower BMI, and why Indian guidelines use a lower weight threshold for risk.

The good news, and the honest caveat

The good news: insulin resistance responds to lifestyle. Losing even a modest amount of weight, moving more, sleeping better and changing the quality of your carbohydrates all improve it. The 2023 international guideline is clear that there is no single magic diet, but that lower-glycaemic eating patterns consistently improve insulin sensitivity, cholesterol and even testosterone.

The honest caveat: you cannot feel insulin resistance. You may feel completely well while your blood sugar and cholesterol quietly drift. That is why testing (section 4) matters more than how you feel, and why this book keeps returning to the same four levers: food, movement, sleep and stress.

Practical marker to watch: your waist. For Indian women, a waist of 80 cm or more signals central fat that drives insulin resistance. Measuring your waist once a month with a tape is a cheap, honest progress check.

7. The long-term risks you are protecting against

PCOS does not only affect your periods. It raises your lifetime risk of several conditions, and managing those risks is a big part of why treatment matters. The WHO and the international guideline list the following as associated with PCOS.

Long-term risk	What you can do about it
Type 2 diabetes and prediabetes	OGTT every 1-3 years; weight, diet and movement; metformin if advised.
High cholesterol and heart disease	Lipid profile at diagnosis and regularly; treat as advised.
High blood pressure	Check at every visit; salt, weight and activity.
Fatty liver	Weight, reduced refined carbs and sugar, alcohol limits.
Sleep apnoea	Report snoring and daytime tiredness; a sleep study if needed.
Endometrial hyperplasia and cancer	Protect the womb lining — regular cycles, weight management, and progestogen therapy where advised.
Depression and anxiety	Screen and treat; these are common and treatable, not a weakness.

Two reassurances. First, the overall chance of developing endometrial cancer is still low — the guideline is careful to say routine screening is not needed, but that you should be told about the risk and about the simple protective steps. Second, most of these risks are exactly the ones that respond to the habits in the second half of this book. You are not fighting a mystery; you are managing a known list.

The guideline also notes something often missed: PCOS is linked to poorer outcomes in pregnancy — higher rates of gestational diabetes and high blood pressure. That is not a reason to fear pregnancy; it is a reason to plan it with your doctor and to test early.

8. Lifestyle is first-line treatment, not a suggestion

This is the single most important sentence in the book: in PCOS, healthy lifestyle is the first-line treatment. The 2023 international guideline recommends lifestyle intervention — diet, movement and behavioural support — for every woman with PCOS, because it improves metabolic health, weight, and often the symptoms themselves.

But "lifestyle" is where most advice goes wrong. It is not about a punishing diet, and it is not about losing a fixed number of kilos. The guideline is explicit that there are benefits even without weight loss, that any sustainable healthy eating pattern works, and that weight should be discussed without stigma. Women with PCOS already face enough judgement.

The four levers, in order of impact

- **Food quality.** Change what your carbohydrates are, before you change how much you eat. Fewer refined carbs and added sugars, more fibre, protein and healthy fats.
- **Movement.** Build towards 150 minutes a week of moderate activity plus strength work twice a week. More is better for weight, but some is far better than none.
- **Sleep.** Seven to nine hours, at consistent times. Poor sleep worsens insulin resistance on its own.
- **Stress.** Chronic stress raises cortisol, which worsens blood sugar and cravings. This is not "just relax" — it is a real lever you will build in section 14.

There is a fifth, quieter lever: behaviour. The guideline recommends goal-setting, self-monitoring, problem-solving and relapse planning because they are what make the other four stick. That is why this book ends with a tracker and a 90-day plan rather than a list of rules.

Set one goal, not five. The guideline recommends SMART goals — specific, measurable, achievable, realistic and time-bound. "Walk 20 minutes after dinner, four nights a week, for one month" beats "get healthy" every single time.

9. The Indian PCOS plate: a practical food system

Here is the honest truth from the evidence: no single diet is proven best for PCOS. The guideline says any healthy eating pattern helps, and the strongest single diet signal in the research is the DASH pattern — high in vegetables, fruit, whole grains, pulses and nuts, low in sodium, saturated fat and added sugar. Lower-glycaemic patterns also consistently improve insulin resistance and cholesterol.

So instead of a foreign diet you cannot follow, this book gives you a way of building Indian meals that does the same job. Think of it as four moves at every meal.

The four moves at every meal

Move	What it looks like on an Indian plate
1. Halve the refined carbs	Less white rice, maida, white bread and sugar. Swap part of the rice for millet, brown rice, or extra dal and vegetables.
2. Add a protein anchor	Dal, rajma, chana, paneer, curd, eggs, fish or chicken — something protein-rich at every meal slows the sugar rise.
3. Add fibre generously	A big portion of vegetables, and a salad. Fibre lowers the glycaemic load of the whole meal.
4. Choose smarter fats and snacks	Nuts, seeds, groundnut or mustard oil in moderation; fruit or roasted chana instead of biscuits and sweets.

Notice what is not here: no banned foods, no "never eat rice". The aim is to change the balance and the quality, not to make eating miserable. A plate you can keep for a year beats a perfect plan you abandon in a week.

Indian foods that work in your favour

- **Millets** — ragi, jowar, bajra, foxtail — and whole pulses are higher in fibre and slower to digest than polished rice.
- **Pulses and legumes** — chana, rajma, moong, masoor, lobia — protein and fibre in one.
- **Vegetables** — gourds, greens, beans, cabbage, brinjal, tomato — fill half the plate.
- **Dairy and eggs** — curd, paneer, milk, eggs — protein and calcium.
- **Nuts and seeds** — almonds, walnuts, flax, chia, pumpkin seeds — in small handfuls.
- **Whole fruit** — guava, apple, orange, papaya, berries — not juice.

Watch these in particular: sweetened drinks and packaged juice, deep-fried snacks, biscuits and bakery items, sweets and desserts, and large portions of white rice at night. These are the biggest glycaemic hits in the typical Indian day.

10. Reading labels and smarter Indian swaps

You do not need to memorise a glycaemic index chart to eat well. You need a handful of swaps that cut the sugar spike without changing your life. And you need to be able to glance at a packet and see what is really in it.

The label, in thirty seconds

- **Look at total sugar per 100 g**, not per serving — serving sizes are chosen to flatter the number.
- **Look at total carbohydrate** and how much of it is sugar. A drink with 12 g sugar per 100 ml is nearly a third sugar.
- **Scan the ingredient list for** sugar, glucose, fructose, corn syrup, maltodextrin, maida and refined flour — these hide the same problem under different names.
- **Prefer foods with more fibre** per 100 g — it slows the sugar release.

Swaps that work in an Indian kitchen

Instead of	Try
Sweetened tea or coffee, several times a day	Less sugar, or unsweetened; herbal or plain milk tea
Packaged fruit juice or cold drinks	Whole fruit or lemon water, chaas, plain water
White bread or maida biscuits	Whole-wheat or multigrain bread; roasted chana or nuts
Deep-fried evening snacks	Roasted chana, sprouts, boiled egg, makhana (small portion)
Large white-rice portion	Half rice plus millet or extra dal and vegetables
Sweet dessert most days	Fruit, or a small dessert on special occasions
Creamy restaurant gravies often	Home-style dal, sabzi and curd-based dishes

None of these swaps is dramatic. That is the point. PCOS responds to the pattern of hundreds of ordinary meals, not to one heroic week. Pick two swaps this week, hold them, then add two more.

11. A 7-day Indian PCOS meal template

This is a template, not a prescription. Portion sizes depend on your body, activity and any advice from your doctor or dietitian. Use it to see the shape of a lower-glycaemic Indian week — then adapt it to your kitchen and your tastes.

Day	Breakfast	Lunch	Evening	Dinner
Mon	Veg poha + boiled egg / curd	2 roti + dal + sabzi + salad	Roasted chana + tea	Millet khichdi + curd
Tue	Besan chilla + mint chutney	Brown rice + rajma + greens	Guava + 6 almonds	2 roti + paneer bhurji + salad
Wed	Oats upma + curd	2 jowar roti + chana + sabzi	Buttermilk + seeds	Moong dal soup + veg + 1 roti
Thu	Idli (2) + sambar	Millet + fish curry + veg	Apple + walnuts	Dal + lauki sabzi + 1 roti
Fri	Vegetable dalia + egg	2 roti + soya chunks curry	Sprouts chaat	Grilled chicken / paneer + veg
Sat	Ragi dosa + chutney	Brown rice + sambar + poriyal	Curd + flax	Veg stew + 1 appam / roti
Sun	Pesarattu + chutney	2 roti + mixed dal + salad	Fruit + nuts	Light khichdi + curd

Rules that make the template work

- Eat protein and vegetables first, then the carbohydrate — it flattens the blood-sugar rise.
- Keep at least a 10 to 12 hour overnight gap between dinner and breakfast where you can.
- Drink water through the day; keep sweet drinks for rare occasions.
- Do not skip breakfast and over-eat at night — that pattern is hard on insulin.
- If you eat rice, pair it with dal, curd and vegetables rather than eating it alone.

If you are vegetarian, the template already leans that way; just make sure every meal has a clear protein anchor — dal, paneer, curd, soya or sprouts. If you eat non-vegetarian food, fish and skinless chicken are easy protein wins.

12. Eating out, festivals and weddings without losing the plot

No PCOS plan survives if it collapses the moment you are at a function. India's festive and wedding season is a marathon of rich food, and this is exactly when a rigid plan fails. The skill is not avoidance — it is damage control you can repeat for the rest of your life.

At a restaurant or a function

- Start with salad, soup or a plain starter so you are not ravenous when the heavy dishes arrive.
- Pick one carbohydrate — not rice and bread and dessert together.
- Choose grilled, tandoori, steamed or curried dishes over deep-fried and cream-heavy ones.
- Fill half the plate with vegetables and dal before the mains.
- Eat slowly and stop at comfortably full, not stuffed.

At a festival or wedding

- Do not "save up" by skipping meals all day — that leads to a bigger spike later.
- Have a protein-rich snack before you go so sweets are a treat, not your whole meal.
- Choose one or two sweets you really love, enjoy them properly, and skip the ones you do not.
- Walk for 15 minutes after the main meal — it blunts the sugar rise.
- Return to your normal plate the very next meal. One day does not undo a month.

The rule that matters most: one celebration cannot hurt you; the week after it can. The single most useful habit in festive season is simply going back to your normal plate at the next meal, without guilt and without a "reset".

13. Movement that fits Indian life

Exercise in PCOS is not only about burning calories. Working muscles pull sugar out of the blood without needing much insulin, and strength training improves how well your body responds to it. The 2023 guideline gives clear, achievable targets.

Goal	The guideline's target
General health	150 to 300 minutes a week of moderate activity, or 75 to 150 minutes vigorous, spread across the week.
Strength	Muscle-strengthening activity on two non-consecutive days a week.
If weight loss is a goal	Up to 250 minutes a week of moderate activity, plus the two strength sessions.
Break up sitting	Replace long sitting and screen time with movement of any intensity.

That sounds like a lot written down, so break it into something real. Thirty minutes of brisk walking, five days a week, is 150 minutes. Two short home strength sessions cover the rest. Add small movement snacks — stairs instead of lift, a walk after dinner — and you are there.

A simple weekly shape

- **5 days:** 30 minutes brisk walk, cycling, swimming or an exercise class you actually enjoy.
- **2 days:** 20 to 30 minutes of strength — squats, push-ups (wall or knee), lunges, planks, and light weights or resistance bands.
- **Every day:** a 10-minute walk after your largest meal.

Yoga is a good addition for stress and flexibility, and many Indian women find it easier to sustain than a gym. It is not a substitute for getting your heart rate up and building strength, but it is a genuinely useful part of the week — especially the calming, breathing-focused styles.

Doing some is better than doing none. If you are starting from zero, begin with 10 minutes, three times a week, and add five minutes each week. Consistency beats intensity — and if you have joint pain, a heart condition or are pregnant, clear your plan with your doctor first.

14. Sleep, stress and the cortisol loop

Sleep and stress are not soft extras in PCOS — they are metabolic. A single short night raises insulin resistance the next day. Ongoing stress keeps cortisol high, which pushes blood sugar up, increases cravings for sugary food, and worsens mood. Fixing sleep and stress often makes the diet and exercise plan work better, not the other way around.

Sleep: the missing treatment

- Aim for seven to nine hours, and keep your sleep and wake times within about an hour each day, including weekends.
- Watch for snoring, gasping or waking unrefreshed — sleep apnoea is more common with PCOS and worth telling your doctor about.
- Keep screens and bright light out of the last hour before bed; keep the room cool and dark.
- Cut caffeine after mid-afternoon.

Stress: build a buffer, not a heroic fix

- Ten minutes of slow breathing, meditation or a walk without your phone, once a day.
- Protect one thing you enjoy each week — it is not a luxury, it is part of the treatment.
- Notice and reduce the "always-on" pattern: constant notifications, work bleeding into rest, no real breaks.
- If low mood or anxiety is persistent, treat it seriously — see section 16.

Why this is not optional: the guideline ranks sleep disorders and psychological features as core parts of PCOS, not side issues. Women with PCOS have more obstructive sleep apnoea than women without it, even at the same weight. If you snore heavily and wake exhausted, that is worth a proper assessment — fixing it can improve energy, mood and blood sugar at once.

One change this week: pick a fixed bedtime and a fixed wake time, and hold them for seven days. Sleep regularity improves insulin sensitivity and appetite control more than most people expect.

15. Hair, skin and weight: the visible symptoms

These are often the symptoms that hurt the most, because they are the ones other people see. They are also the ones with real, evidence-based options — so there is no reason to simply live with them.

Unwanted hair (hirsutism)

Options work best in combination, and take months, not weeks. Mechanical removal (threading, waxing, shaving) is quick and safe. Laser and light-based therapies are effective for longer-term reduction — a 2024 review found the alexandrite laser worked better than intense pulsed light, and that combining laser with the pill or metformin improved results. Medical options (the combined pill, and anti-androgens if needed) lower androgen levels and are covered in section 17. The 2023 guideline now includes laser as a recommended option for hirsutism in PCOS.

Acne and scalp hair loss

Persistent acne on the jaw, chest or back that resists ordinary creams is often androgen-driven and responds to the combined pill or to dermatology treatment. Scalp thinning is distressing but treatable; ask your doctor about options rather than waiting. In both cases, treat the underlying androgen excess, not just the surface.

Weight — handled with honesty and without shame

Weight sits at the centre of many PCOS symptoms, and this is where women are most often treated badly. The guideline is direct: many women with PCOS experience weight stigma, and it harms care. You are allowed to want weight change for your health without accepting judgement. The realistic goal is not a number on a chart but a lifelong pattern — preventing further gain, improving waist size and metabolic health, and losing a modest amount where it helps your symptoms.

> **Ask for care, not comments.** If a clinician talks about your weight in a way that shames rather than helps, you are entitled to say so and to ask for a plan that focuses on health behaviours. Stigma is not treatment.

One practical point on the visible symptoms: they tend to improve in the same order you might expect, but slowly. Cycle regularity and metabolic markers often respond first, within a few months of consistent lifestyle change. Hair changes take the longest — expect six months or more, because hair grows in long cycles and androgen effects unwind gradually. Knowing this ahead of time stops the discouragement that makes people give up just before things start to shift.

16. Mental health and PCOS — you are not alone

The 2023 guideline makes one thing unmistakably clear: depression and anxiety are very common in PCOS, in both adults and adolescents, and every woman with PCOS should be screened for them. This is not you being dramatic. It is a documented part of the condition, and it is treatable.

PCOS can affect mood through several routes at once — hormonal changes, the daily stress of symptoms, body-image distress, and sometimes the pain of fertility problems. Disordered eating and body image struggles are also more common, which is why any weight advice must be gentle. The guideline recommends psychological support — including cognitive behavioural therapy — where these problems are present, and treatment for depression or anxiety where it is diagnosed.

If you are struggling, here is what helps

- Tell your doctor how you are actually feeling — mood is part of the medical review, not a separate issue.
- Ask to be screened for depression and anxiety; validated questionnaires exist and are quick.
- Ask about psychological therapy, which the guideline supports.
- Be honest about eating patterns — if food has become a source of fear or control, say so; it is common and it is treatable.

Free, confidential help in India: the Government's Tele-MANAS helpline is open 24x7 on **14416** (or 1-800-891-4416). If you ever have thoughts of harming yourself, please reach out to it or to someone you trust right away. You deserve support, not silence.

It is also worth saying plainly: living with a chronic condition is tiring, and feeling low about it is a normal human response, not a failure of character. Naming that feeling — to a doctor, a counsellor, a friend or a family member — is one of the most useful things you can do for your PCOS.

17. Medical treatment options your doctor may offer

This section explains what medicines are commonly considered in PCOS and why, so you can have an informed conversation. It is not a prescription. Only your doctor can decide what is right for you, and some of these uses are off-label — meaning the medicine is evidence-based for PCOS but not formally approved for it, which your doctor should explain.

Treatment	What it is used for
Combined oral contraceptive pill (COCP)	First-line for irregular cycles and for androgen-related symptoms such as hirsutism. Low-dose oestrogen preparations are preferred.
Metformin	Recommended mainly for metabolic features — weight, insulin resistance, blood sugar and lipids — especially with a BMI of 25 or above. Mild stomach upset is common and usually settles.
Letrozole	First-line medicine to induce ovulation if you are trying to conceive.
Clomiphene, or gonadotrophins	Other ovulation-induction options, used in sequence if needed.
Anti-androgens	Considered for hirsutism if the pill and cosmetic therapy have not worked after at least six months; always with effective contraception.
GLP-1 medicines (e.g. semaglutide)	May be considered alongside lifestyle for weight management, per general guidelines, with careful discussion of side effects and long-term use.
Progestogens	To protect the womb lining when periods are very infrequent.
Laser / light hair reduction	Recommended for hirsutism; works best combined with medical treatment.

Two points worth remembering. First, metformin and active lifestyle change have similar effectiveness for many metabolic outcomes — the pill is not a substitute for the habits in this book, and the habits are not a substitute for the pill where it is needed. Second, fertility medicines should be managed by a specialist; do not take ovulation-inducing drugs without supervision.

A word on how to choose with your doctor. The guideline recommends shared decision-making: your priorities should decide the plan. If your biggest concern is acne, that points one way; if it is a future pregnancy, another; if it is metabolic risk, another again. Say what matters most to you at the start of the consultation.

18. Supplements: what the evidence actually says

PCOS is a huge market for supplements, and much of what is sold has weak evidence behind it. Here is an honest reading of the research that informed the 2023 guideline — so you spend your money where it might help, and not where it will not.

Supplement	What the evidence says
Inositol (myo- and D-chiro-)	Widely sold and popular, but the evidence is limited and inconclusive. It may help some metabolic measures and cause fewer stomach side effects than metformin, but metformin was superior for central fat and hirsutism. Not a proven replacement.
Vitamin D	Worth checking if you are deficient — common in India — and correcting as advised; it supports overall metabolic health.
Metformin	Not a supplement, but the evidence-based insulin sensitiser; see section 17.
Multivitamins, "hormone balancers", detox teas	No reliable evidence in PCOS. Be sceptical of strong claims.

The honest rule: supplements are not regulated the way medicines are, and they cost real money. Before buying anything, ask your doctor whether there is evidence for it in your case. Correcting a genuine vitamin D deficiency is worthwhile; chasing a marketed "cure" usually is not.

The guideline is unusually blunt on this point: it says the evidence on inositol is inadequate to recommend it for clinical outcomes, and that the finding should be used to limit potential harm — including financial harm — to women buying therapies without proven benefit. That is worth remembering the next time an ad promises to "balance your hormones".

19. Your lab report decoder

Reports are written for doctors, not patients, so here is a plain guide to the numbers that matter most in PCOS. These are general adult reference points — your lab's own ranges and your doctor's reading are what count, especially around pregnancy, other conditions and medication.

Test	General adult guide	What to watch
Fasting glucose	Below 100 mg/dL normal; 100-125 prediabetes; 126 and above (on two tests) diabetes	A rising trend over years matters even inside the normal band.
HbA1c	Below 5.7% normal; 5.7-6.4% prediabetes; 6.5% and above diabetes	Shows your average blood sugar over about three months.
Total cholesterol	Below 200 mg/dL desirable	Read with LDL, HDL and triglycerides, not alone.
LDL cholesterol	Below 100 mg/dL desirable	The main artery-clogging fraction.
HDL cholesterol	Above 50 mg/dL for women is protective	Higher is better; activity raises it.
Triglycerides	Below 150 mg/dL desirable	Strongly tied to refined carbs and sugar.
TSH	Roughly 0.4-4.0 mIU/L, lab-specific	Out of range suggests a thyroid problem that mimics PCOS.
Prolactin	Lab-specific; often under about 25 ng/mL	Raised levels need a look for other causes.
Testosterone / FAI	Lab-specific	Interpreted with your symptoms and the free androgen index.
Waist	Under 80 cm for Indian women	A simple, cheap marker of metabolic risk.
BMI (Asian Indian)	Overweight from 23; obese from 25	Indian thresholds are lower than Western ones.

Keep a one-page summary of your numbers with the date beside each. At your next visit, you can see change at a glance — and ask better questions. A number without a trend tells you very little.

20. PCOS and other conditions to watch

PCOS rarely travels alone. Several other conditions are more common in women with PCOS, and some look so similar that a good diagnosis has to rule them out first. Knowing them helps you ask the right questions.

Conditions that can look like PCOS

- **Thyroid disorders** — can cause irregular periods, weight change and fatigue; a TSH test screens for them.
- **High prolactin** — can cause missed periods and, sometimes, milk discharge; screened with a prolactin test.
- **Non-classic congenital adrenal hyperplasia** — a genetic cause of high androgens; considered if androgens are very high.
- **Cushing's syndrome** — excess cortisol; suspected with rapid weight gain, stretch marks and a rounded face.
- **Androgen-secreting tumours** — rare, but suspected with rapid or severe virilising symptoms.

Conditions that commonly travel with PCOS

- **Insulin resistance and prediabetes** — the most common companion; tested with an OGTT.
- **Fatty liver** — linked to insulin resistance; about one in three Indian women with PCOS in the ICMR study.
- **High cholesterol** — present in the large majority of Indian women with PCOS.
- **Anaemia and vitamin D deficiency** — very common in Indian women generally; worth checking and correcting.
- **Sleep apnoea** — more common with PCOS, even at the same weight.

Why this matters: if a symptom does not fit the PCOS picture — rapid weight gain, very high androgens, or unusual fatigue — it is worth asking whether something else is going on alongside it. Good care rules things out rather than assuming.

21. PCOS and fertility: trying to conceive

PCOS is a leading cause of difficulty getting pregnant, because ovulation is irregular. But the message to hold on to is this: most women with PCOS who want to conceive are able to, often with simple support. PCOS reduces the frequency of ovulation; it does not usually take it away completely.

What helps, in order

- **Lifestyle first.** Even modest weight loss and better insulin sensitivity can bring back regular ovulation — this is the foundation of fertility care in PCOS.
- **Letrozole** is the first-line medicine to induce ovulation, with clomiphene and metformin also having a role.
- **Gonadotrophins** are a second-line option, and **IVF** a third-line option where other methods have not worked.
- **Pre-conception checks:** the guideline advises a glucose test before pregnancy and again in the second trimester, because gestational diabetes is more common with PCOS.

Two practical notes for India. First, ask for a pre-conception review rather than waiting — it lets your doctor optimise your blood sugar, blood pressure and medicines before you conceive. Second, ovulation-inducing medicines carry a risk of multiple pregnancy and must be monitored; please do not use them without a specialist's supervision.

You are not out of time. PCOS does not usually cause early menopause, and ovarian reserve is often normal or even high. A diagnosis of PCOS is not a verdict on your fertility — it is a reason to plan, test and treat early.

A note on the emotional side of this section, because it is real. Difficulty conceiving is one of the heaviest experiences a person can carry, and in India it often comes with family pressure and unsolicited advice. Please know two things: you are allowed to find it hard, and you are allowed to ask for support — medical, psychological, and from the people you trust. Fertility care works best when your wellbeing is treated as part of the plan, not an afterthought.

22. PCOS in adolescents and across the family

PCOS often begins in the teenage years, and India has a high burden among adolescent girls — FOGSI estimates roughly one in five. But diagnosing it in adolescence is genuinely difficult, because irregular periods and acne are normal in the first years after menarche.

Why the rules are stricter for teenagers

- An ultrasound is not used to diagnose PCOS in adolescents — multi-follicular ovaries are normal at this age.
- Both irregular cycles *and* signs of high androgens are needed before a diagnosis is made.
- A girl with some features but not enough for diagnosis is called "at increased risk" and reassessed later — often at or before full reproductive maturity, about eight years after her first period.

The focus in adolescence is healthy lifestyle and preventing excess weight gain, plus protecting emotional wellbeing. Early acne or hirsutism, a recent belly-weight gain, a sedentary routine and a family history of diabetes or PCOS are the signals FOGSI says should prompt a closer look.

PCOS runs in families — so does the risk

PCOS has a strong genetic component, and fathers and brothers of women with PCOS may have higher rates of metabolic syndrome, type 2 diabetes and high blood pressure. If one woman in a family is diagnosed, it is worth the family knowing — not to alarm anyone, but so that screening and healthy habits start early.

For parents: the most useful things you can do are to avoid weight-shaming, keep the whole family's food and activity habits healthy rather than singling out one daughter, and make sure she has a doctor she can talk to. PCOS in a teenager is a reason for care, not blame.

23. Your doctor-visit toolkit: tests, questions, tracker

A good consultation is a partnership. This section gives you three things to take with you: the tests worth asking about, the questions worth asking, and a simple tracker.

Tests to discuss

Testosterone (total / free androgen index), TSH, prolactin, FSH and LH, an optional AMH, a 75g oral glucose tolerance test, fasting glucose and HbA1c, a fasting lipid profile, blood pressure, and a pelvic ultrasound in adults. See section 4 for what each one is for.

Questions worth asking

Ask this	Why it helps
Which criteria do I meet, and which phenotype am I?	Tells you what your PCOS is actually driven by.
What are my glucose, cholesterol and blood-pressure numbers?	These drive your long-term risk, more than symptoms do.
Do I need to be screened for depression or anxiety?	The guideline recommends it for everyone with PCOS.
What is my realistic first goal, and how will we measure it?	Turns vague advice into a plan you can track.
Are the medicines you suggest off-label for PCOS?	You deserve to know the evidence and the reasons.
When should I come back, and what should have changed by then?	Sets a clear follow-up, usually 6 to 12 months.

A 6-month tracker you can keep

- Period dates and how heavy each one was.
- Waist measurement, once a month.
- Weight, if you choose to track it — but judge the trend, not one day.
- Energy and mood, rated simply (good / okay / low).
- Any new hair, skin or sleep changes.
- Your latest blood-test numbers, in one place.

None of this needs an app. A single page in a notebook, filled in once a month, will do more for your care than any gadget — because it turns your experience into evidence your doctor can act on.

24. Myths vs facts

PCOS is surrounded by confident nonsense. Here is a short correction, so you are not talked into or out of something important.

Myth	Fact
"PCOS means you have ovarian cysts."	The bumps are immature follicles, not true cysts, and you can have PCOS without them.
"PCOD and PCOS are the same thing."	"PCOD" is a loose label, not a formal diagnosis; PCOS is the recognised condition with defined criteria.
"You must give up rice and eat only salads."	No single diet is proven best. Change the quality and balance of your carbohydrates; keep a plate you can sustain.
"PCOS means you cannot have children."	PCOS reduces ovulation frequency but most women conceive, often with simple support.
"Losing weight cures PCOS."	There is no cure. Weight change helps many symptoms, and healthy lifestyle helps even without weight loss.
"You are just not trying hard enough."	PCOS has a strong genetic and metabolic basis. It is not a willpower problem, and shame does not help.
"A supplement will fix your hormones."	Most supplements have weak or inconclusive evidence. Correct genuine deficiencies; be sceptical of cures.
"PCOS goes away after marriage or pregnancy."	Symptoms can change with life stage, but PCOS is lifelong and needs ongoing care.

25. Your 90-day action plan

You do not need to change everything today. This plan starts with one visit and one habit, then adds the next only when the last one is holding. Ninety days is enough to see real movement in energy, cycles and how you feel.

Days 1 to 14 — get the facts

- Book a doctor's visit; take sections 4 and 23 with you.
- Start tracking your period dates and one weekly waist measurement.
- Pick the single food change with the biggest effect for you — most often cutting sweetened drinks and late white rice.

Days 15 to 45 — build the base

- Move to the four-moves plate at most meals (section 9).
- Walk 30 minutes, five days a week; add two short strength sessions.
- Fix your sleep and wake times, and protect seven hours.
- Add one daily de-stress block of ten minutes.

Days 46 to 90 — make it stick

- Re-check your numbers at the follow-up your doctor sets.
- Review what is working and drop what is not — keep the smallest version of each habit you can maintain.
- If you are on treatment, keep taking it as prescribed and report side effects instead of stopping silently.
- Set your next 90-day goal, and keep going. PCOS is a long game played gently.

The whole book in one line: test properly, eat and move kindly, sleep and de-stress deliberately, take medicine only as prescribed, and treat your mood as part of your health — and you will manage PCOS far better than the condition manages you.

Quick-reference card

If you remember nothing else from this book, remember this page. It is the whole system, compressed.

The four levers	The target to build towards
Food	Four-moves plate: fewer refined carbs, protein and fibre at every meal, smarter snacks.
Movement	150 min/week moderate activity + strength twice a week; a 10-min walk after the main meal.
Sleep	7-9 hours, fixed times, no snoring ignored.
Stress	10 minutes of calm daily; persistent low mood gets treated, not endured.

Check	How often
Period dates, energy and mood	Every cycle, in one notebook page.
Waist measurement	Once a month (target: under 80 cm).
Blood sugar (OGTT), cholesterol, blood pressure	At diagnosis and every 1-3 years, or as advised.
Depression and anxiety screening	At diagnosis, then as your doctor advises.

Remember: test properly, eat and move kindly, sleep and de-stress deliberately, take medicine only as prescribed, and treat your mood as part of your health. Free mental-health support in India: Tele-MANAS on 14416, 24x7.

Frequently asked questions

Will I have PCOS for life?

Yes. PCOS is a chronic condition that continues past the reproductive years, though symptoms change with age. It is very manageable, and treatment reduces both symptoms and long-term risk.

Do I need to lose weight to improve my symptoms?

Not necessarily. The guideline notes benefits from healthy lifestyle even without weight loss. If you do carry extra weight, modest loss and a smaller waist often improve cycles and metabolic markers — but the goal is health, not a number.

Is it safe to get pregnant with PCOS?

Usually yes, with planning. PCOS is linked to higher risk of gestational diabetes and high blood pressure, so a pre-conception review and monitoring in pregnancy matter. Most women with PCOS have healthy pregnancies.

Can I stop my medicine once my periods become regular?

Only with your doctor's advice. Regular cycles on treatment often mean the treatment is working, not that it is no longer needed. Never stop prescribed medicine on your own.

Is intermittent fasting or keto good for PCOS?

There is no strong evidence that any one diet is best. Very restrictive diets are hard to sustain and can harm mood and eating habits. A flexible, lower-glycaemic pattern you can keep works better.

Does PCOS cause infertility permanently?

No. PCOS is a leading cause of difficulty conceiving, but it reduces ovulation rather than removing it. Most women conceive, often with simple ovulation support.

Are the numbers in this book India-specific?

Yes. Prevalence and metabolic figures come from the ICMR national PCOS study and FOGSI's India recommendations, and the management advice follows the 2023 international guideline that Indian clinicians use.

Can teenagers get PCOS?

Yes, and it often starts in the teenage years, but diagnosis is deliberately cautious at that age. An ultrasound is not used, and both irregular cycles and high-androgen signs are needed. Girls with some features are followed up as "at increased risk".

Sources and further reading

Every fact in this playbook is drawn from the official and peer-reviewed sources below. Where a figure is quoted, it comes directly from these documents.

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Thank you for reading. This playbook exists because too many Indian women are handed a one-line diagnosis and no plan. You now have the plan. Take it to your doctor, start with one habit, and be patient with your body.